

FC-002: James Okafor — Interview Transcript

Interview Transcript

PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2 (Synthetic Interview Data)	Participant ID	FC-002
Persona	Family Caregiver	Next Step	Feed into `/summarize-interview` with other FC interviews

SESSION DETAILS

Date	2026-03-29	Duration	38 min
Interviewer	PM Lead	Setting	Face-to-face
Location	James's home office (granny flat visible through window)		

PART A — Narrative Transcript

Opening & Warm-Up

Q: Tell me about your role --- what does a typical week look like caring for your father?

My father Emmanuel is eighty-one. Early-stage dementia, Type 2 diabetes, hypertension. Seven medications. He lives in the granny flat attached to our house — about ten steps from our back door. I work from home as an IT consultant, so I'm nearby all day. My wife Adama helps with daily care — she checks on him at lunch, makes sure he's eaten. We're both doing this full time on top of our actual jobs.

Block 2: Medications

Q: Tell me about the last time there was a problem with a medication.

Last month. My father took his metformin twice in one morning. I fill the weekly organiser every Sunday. He opens it, takes the morning pills, closes it. But that day — I don't know if he forgot he'd taken them or if he thought it was the afternoon dose — he opened it again and took the same dose. His blood sugar crashed. He was sweating, confused, couldn't stand. I thought he was having a stroke. We went to emergency. Sat there for six hours. They gave him glucose, monitored him, adjusted his dose. The geriatrician said we should look into a medication management service. But nothing has actually changed about the daily routine. It's still me and the pill organiser and hope.

[Probe] What information did you wish you had?

Something that catches the double dose before it happens, not after. I don't need a reminder — he knows he needs to take pills. I need an interceptor. Something that says 'This dose was already taken thirty minutes ago' before he opens that compartment a second time.

Block 1: Falls

Q: Has your father had any falls or near-falls?

He hasn't fallen yet. But I can see it coming. He gets up from his chair too quickly and wobbles. He shuffles more than he used to. The granny flat has a step between the living area and the bathroom that I've been meaning to get ramped for months. He won't use a walking frame — same pride issue as the medication thing. He was a pharmacist for forty years. He sees himself as the expert, not the patient. The cruel irony is he

filled prescriptions for other people his whole career. He knew every interaction, every contraindication. Now he can't remember if he took his own pills twenty minutes ago. The look on his face when I have to ask him — it's humiliating for both of us.

[Emotional note] James paused for 15 seconds here. Longest silence in the interview. Processing.

Block 3: Communication & Escalation

Q: How does information about your father's health get to his geriatrician?

I bring handwritten notes to the quarterly appointment. My best guess at what happened over three months. I write down things as they occur — 'Tuesday: seemed more confused than usual,' 'Friday: didn't finish dinner,' 'Monday: took metformin twice.' But by the appointment, I've lost half the details and the ones I kept might not be the ones that matter clinically. I want the geriatrician to see real data. Not my scribbled notes from three months of guessing. Actual patterns. What's his blood sugar been doing? How many doses did he actually miss? When does the confusion peak — morning or evening? I can't track that manually with enough consistency.

Block 4: Tools & Workarounds

Q: What tools do you currently use?

Pill organiser. Whiteboard in his kitchen with the daily schedule. A baby monitor at night — he doesn't know about that one, and I feel guilty every time I look at it. My wife checks at lunch. iPhone calendar reminders for his appointments. Handwritten notes for the geriatrician.

Q: Have you tried any technology solutions?

Two medication reminder apps. Both failed. Big flashing buttons. Loud alarms. Patronising voice messages: 'It's time to take your medication!' My father is a retired pharmacist, not a toddler. He looked at the first one and said, 'Who designed this, someone who's never met an adult?' He's right. If the technology looked like a clinical tool instead of a toy, he'd actually use it.

Block 5: Willingness to Prioritize

Q: How much time per week do you spend on care coordination?

Twelve hours, minimum. Plus the emotional overhead. My wife Adama said to me last week, 'I love your dad, but I can't keep doing this.' That's when I knew we need systematic help. Not another app that beeps. Real help.

Q: What would a fair price look like?

Two hundred, two-fifty a month. Without hesitation. The ER visit from the double dose probably cost the healthcare system five thousand dollars. One tool that prevents that pays for itself in a single incident.

Wrap-Up

Q: Who else should I talk to?

My father. But approach him as a former healthcare professional, not as a patient. Ask him what he would have designed when he was a pharmacist. He'll engage with that conversation. He won't engage with 'How do you feel about your memory?'

PART B — Filled Note Template

Participant ID	FC-002
Persona	FC

Date	2026-03-29
Duration (min)	38
Interviewer(s)	PM Lead
KEY JOBS	Prevent medication errors that could hospitalise father. Maintain dignity and independence. Provide accurate longitudinal data to geriatrician. Keep marriage intact under caregiving burden.
CURRENT SOLUTION	Weekly pill organiser. Kitchen whiteboard. Baby monitor (secret). Wife checks at lunch. iPhone calendar. Handwritten notes for quarterly geriatrician. Previously: 2 medication apps (both rejected for being patronising). "Last month he took his metformin twice. His blood sugar crashed. We sat in emergency for six hours. All because of one extra pill." "My wife said, 'I love your dad, but I can't keep doing this.'" "I want the geriatrician to see real data, not my scribbled notes from three months of guessing."
WILLINGNESS	12+ hrs/week. Would pay \ \$200-250/mo. ER visit cost \~\ \$5,000 — one prevented incident pays for the tool. Rejected 2 apps for patronising design. Key: must feel like a clinical tool, not a toy.
EMOTIONAL INTENSITY	15-second silence when describing role reversal. Deep sadness about father's lost professional identity. Strained marriage was most emotionally charged moment. Guilt about secret baby monitor.
ESCALATION BEHAVIOR	Checks pill organiser 3x daily (8 AM, noon, 6 PM). Called geriatrician after-hours twice in 6 months: once for double-dose, once for 2-day medication refusal. Doesn't know when to push through father's resistance vs escalate.
SURPRISE FINDING	Father will engage with technology that feels 'professional' — he reads pharmaceutical journals on his iPad. Apps that failed were patronising. A clinical interface would get adoption. Also: dignity-preserving design is not a feature — it's a prerequisite.
TOOL MENTIONS	Pill organiser. Whiteboard. Baby monitor. 2 medication apps (rejected). iPhone calendar.
REFERRAL	Emmanuel (father) — 'approach him as a former healthcare professional.' The geriatrician.
FOLLOW-UP ACTIONS	[] Explore 'interceptor' concept for double-dose prevention [] Design for dignity: clinical aesthetic, not consumer toy [] Quantify ER cost avoidance as value proposition